

Situation Report of the 17th Ebola Virus Disease (EVD) Outbreak / DRC
SitRep MVE No. 006/MVB_17/2026

Affected provinces	ITURI, NORTH KIVU
Affected Health Zones	ITURI : Mongbwalu, Rwampara, Bunia, Nyankunde, NORTH KIVU : Goma, Katwa and Butembo
Reporting date	May 20, 2026
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Distribution of confirmed cases and deaths by health zone and province

Affected provinces	Health zones with case confirmed	New case confirmed	New deaths confirmed	New case probable	New case suspects	New contacts listed	Healed by the day
Ituri	Bunia	5	0	ND	35	0	0
	Rwampara	3	0	ND	25	286	0
	Mungbwalu	0	0	ND	8	0	0
	Bamboo	1	0	ND	1	128	0
	Nyankunde	3	0	ND	9	0	0
Subtotal		12	0	0	78	414	0
North Kivu	Goma	0	0	ND	ND	ND	0
	Butembo	0	0	ND	ND	ND	0
	Katwa	1	0	ND	ND	ND	0
Subtotal		1	0	0	0	0	0
Total		13	0	0	78	414	0



High-level meeting held
by the Minister of
Communication with the
Minister of Health, the Director
General of Africa CDC, Muyembe
and the Director General of INSP

I. CONTEXT

The Ituri province is located in the northeast of the Democratic Republic of Congo and shares a long border with Uganda and South Sudan. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements.

It has an estimated population of over 8 million inhabitants, including more than one million internally displaced persons.

In September 2018, this province was affected by an Ebola virus disease epidemic that was raging in the neighbouring province of North Kivu.

The Mongbwalu health zone is one of the 36 health zones in Ituri province. Considered the starting point of the epidemic, it is located in the Djugu territory, where several armed groups operate. It is situated 70 km from Bunia, the capital of Ituri province, and experiences frequent population movements towards Uganda. This area is characterized by numerous construction projects that attract people from other territories within the province and from neighboring provinces.

II. HISTORY OF THE EPIDEMIC

III.

On May 5, 2026, social media reported a surge in deaths of unknown cause, with nearly 50 deaths already recorded in the Mongbwalu health zone. Following confirmation of this alert, the Chief Medical Officer of the Mongbwalu health zone shared the preliminary investigation report on May 9, 2026. On May 11, 2026, provincial health authorities held an emergency meeting and decided to deploy rapid response teams to Mongbwalu and Rwampara, which had reported the outbreak.

suspected deaths from viral hemorrhagic fever among patients interned at the Evangelical Medical Centre of Rwampara, coming from Mongbwalu.

Investigations conducted on May 12, 2026, simultaneously in the Mongbwalu and Rwampara health zones by the Rapid Intervention Teams of the Ituri Provincial Health Division (DPS), revealed that most suspected patients or deaths presented with fever, headache, vomiting, and severe physical asthenia. Deaths occurred 2 to 3 days after the onset of illness, with symptoms including hematemesis, epistaxis, or blackish vomit. During the investigations in the Rwampara health zone, 20 samples were collected from suspected cases or deaths. The majority of samples were taken from the Mongbwalu health zone. Preliminary tests conducted at the Bunia Laboratory were all negative for Ebola Zaire, dengue fever, rotavirus, cholera, malaria, *Yersinia pestis*, measles, and COVID-19. These samples were sent to the National Institute of Biomedical Research (INRB) in Kinshasa for further testing.

As of May 14, 2026, **13 of these 20 samples were tested, and 8 were found to be positive** for non-Zairean Ebola virus disease at the INRB in Kinshasa. On May 15, 2026, sequencing of the positive samples identified the BUNDIBUGYO strain, *allowing the Minister of Public Health, Hygiene and Social Welfare (MSHPS) to officially declare the 17th Ebola virus disease outbreak.*

Ebola virus disease (Bundibugyo strain), thus marking the 17th epidemic in the country's history

As of May 17, 2026, in consultation with member states, the WHO Director-General considers that the event taking place in DRC and Uganda meets the criteria of the definition set out in Article 1 of the IHR, and declared that this constitutes a Public Health Emergency of International Concern while specifying that the risk at the global level is low but high at the regional level.

On May 18, 2026, the Director General of the Africa CDC officially declared a Continental Security Public Health Emergency (CSPHE) under Article 3 of its statutes. This declaration followed an emergency meeting of the Africa CDC Advisory Group, which determined that the worsening outbreak posed an acute risk to continental health security. Consequently, the continental team

CHRONOLOGIE DES EVENEMENTS

2014

24 août 2014 Le deuxième cas suspect (fraternel d'un) a été développé des symptômes. Transmission intra familiale prouvée.

26 août 2014 Le premier cas suspect a été déclaré. Forte mortalité ; syndrome hémorragique possible.

27 août 2014 Décès du deuxième cas suspect. Progression rapide et regroupement et (cluster) d'autres fatales.

2015

04 mai 2015 Alerte mondiale, en attente de vérification.

05 mai 2015 Test GenScript négatif à Ebola. Échantillons envoyés à l'WHO Kinshasa.

06 mai 2015 Alerte sur les réseaux sociaux : 50 décès à Mungahulu.

13 mai 2015 Début de la vérification de l'alerte par les autorités sanitaires.

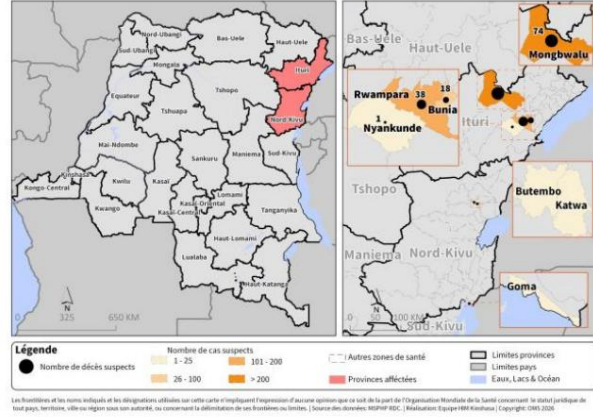
14 mai 2015 Partage du premier rapport d'investigation (Mondzhalu & Bwena). Réunion nationale du COUSP (Centre d'Opérations d'urgence de Santé Publique) convoquée.

15 mai 2015 L'WHO Kinshasa confirme le virus Ebola (après mon-Zaire). Échec à identifier le patient ; Zaire écarté.

2016

15 mai 2016 L'Organisation mondiale de la Santé confirme le virus Ebola (après mon-Zaire). Échec à identifier le patient ; Zaire écarté.

15 mai 2016 Le séquençage génomique confirme le virus Ebola (après mon-Zaire). Échec à identifier le patient ; Zaire écarté.



II. EPIDEMIOLOGICAL SITUATION UPDATE

- ➔ A new area has been affected in the province of Ituri, namely Bambu.
- ➔ The cumulative number of confirmed cases is 64, including 60 in the Ituri DPS (8 affected health zones) and 4 in North Kivu (3 affected health zones).
- ➔ Listing of 414 new contacts, including 286 in the Rwampara health zone and 128 in the Bambu health zone.
- ➔ No new deaths confirmed. The total number of deaths is 6, including 2 in North Kivu and 4 in Ituri.

III. RESPONSE ACTIONS

COORDINATION

- Holding of a coordination meeting chaired by the Deputy Incident Manager, Head of the Provincial Health Division of North Kivu (Beni) and Ituri (Bunia).
- Holding of the partner mobilization meeting on alignment with the response plan.
- Development of the provincial action plan for the Ebola Virus Disease incident in the DPS Ituri and North Kivu.

EPIDEMIOLOGICAL SURVEILLANCE

- ➔ Support to the Katwa and Goma health zones in the in-depth investigation and listing of contacts around confirmed cases.
- ➔ Implementation of an alert management system (publicizing the civil protection hotline number).
- ➔ Contact tracing has begun around the confirmed case in the Goma health zone and activation is underway in the Ituri health zones.
- ➔ Alert note from the PoE correspondent in Ntoronko/Uganda regarding the risk of contamination by Congolese residents in Uganda returning from the mourning grounds for their deceased brothers in the DRC on the shores of Lake Albert.
- ➔ In-depth investigations and contact tracing around 4 confirmed cases in North Kivu and 58 cases in Ituri.

Table I. Summary of the alert status as of May 20, 2026

Monitoring of alerts	Identified	Alerts											
	0	39				35	4	35	0	11			1
	4	31				35	0			14			0
	0	9	9	9	0	9	0	9	0	9	0	9	0
	0	9	9	6	3		1	8	1	7	1	6	0

Table II. Update of the Ebola Virus Disease Epidemiological Situation in the Province of Ituri as of 20/05/2026

[illegible]

Table III. Summary of contact identification and follow-up as of May 20, 2026

[illegible]

Bunia	203	0	203	0	0	0	203		0	0	0	0	0	200	203
Rwampara	335	286	621	83	0	0	14	538	0	0	0	0	0	538	621
Nyankunde	133	0	133	0	0	0	133		0	0	0	0	0	133	133
Mongbwalu	176	0	176	0	0	0	176		0	0	0	0	0	176	176
Bamboo	0	128	128				0% 128		0	0	0	0	0	128	128
Total	847	414	1261	83	0	0	7% 1178		0	0	0	0	0	1178	1261

Table IV. Distribution of cases, suspected and confirmed deaths of Ebola virus disease in the Health zones of the DPS Ituri and North Kivu as of May 20, 2026

Provinces	Health zones	Number of suspected cases	Number of suspicious deaths	Number of cases Confirmed	Number of contacts
Ituri	Mongbwalu	317	80	13	176
	Rwampara	179	44	24	621
	Bunia	125	31	11	203
	Nyankunde	28	5	5	133
	BAMBU	2		1	128
	Samples without a datasheet	0 4 0 1 13	1 7 1 671 61	*These data are	0
North Kivu	Goma	subject to change after harmonization of			N
	Katwa	data at different levels of the health			ND
	Butembo	pyramid with the incident management			ND
Total		system.	0 0 0 0 160		1261

Point of Entry and Point of Control (PoE/PoC) Surveillance

- ➔ Activation and operationalization of the PoC Dele (ZS Rwampara) and Mudzibala (ZS Bunia) with support from IOM.
- ➔ Identification of resources by PoE/PoC (124 providers for 10 PoCs).
- ➔ Donation of 120 plastic chairs, 60 boxes of gloves and other PCI materials (liquid soaps etc) by the IOM.
- ➔ Status of the PoEs of Kasindi, Nobili and evaluation of the PoCs of Eringeti, Mavivi Airport, PK5 and Nzuma.

Table V. Data summary (PoE/PoC) as of 20/05/2026

Health Zone	PoE/PoC	Tot v past	Tot v screened	Total washing of hands	Tot v sensitized	Early alerts	Total refusal Handwashing	Total refusal screening
NIZI	NIZI BRIDGE	0	0	0	0	0	0	0
BUNIA	AIRPORT 189		189	189	189	0	0	0
	MUDZIBALA 355		355	355	355	0	0	0
	BRIDGE SIMBILIABO	0	0	0	0	0	0	0
RWAMPARA	SHARI BRIDGE 0		0	0	0	0	0	0
	DELE	102	102	102	102	0	0	0
	CHAI	0	0	0	0	0	0	0

TCHOMIA	TCHOMIA 105		105	105	0	0	0	0
	NYAMUSASI 0		0	0	0	0	0	0
	KASENYI	186	186	186	186	0	0	0
	MANDJE	0	0	0	0	0	0	0
NYAKUNDE	MARABO 0		0	0	0	0	0	0
	ENTRANCE NYAK.	0	0	0	0	0	0	0
	SOTA	0	0	0	0	0	0	0
	BRIDGE TALOLO	0	0	0	0	0	0	0
MUNGWALU	POIPO	0	0	0	0	0	0	0
	MAHIDEDE 0		0	0	0	0	0	0
	PLITO 0 BRIDGE		0	0	0	0	0	0
	BRIDGE KIRIKOU	0	0	0	0	0	0	0
ARU	OFOO	195	195	195	195	0	0	0
	ODHUMONI 146		146	146	146	0	0	0
	OMBAYI	253	253	253	253	0	0	0
	WITHOUT COMPLAINT	124	124	124	124	0	0	0
MAHAGI ANZIDHA		184	184	184	184	0	0	0
TOTAL		1881	1881	1881	1881	0	0	0

LABORATORY

Samples received on 20/05/2026

- Samples analyzed: 35 Positive
- results for the day: 12 (Rwampara 3; Bunia 5; Nyakunde 3 and Bambu-, mine 1)
- Negatives of the day: 13
- Disabled persons of the day: 10 • Sample reports: 49
- Samples received: 92 •
- Total samples to be analyzed: 141
- 102 samples sent to Kinshasa on 20/05/2026 for sequencing.

Results as of May 20, 2026 (RADIONE)

- Total samples received: 218
- Re-samples (control): 3, of which 2 were positive and 1 was not analyzed
- Total samples analyzed: 134
- Positive totals: 64



16 samples were received at the Beni Laboratory, of which 7 have already been sent to Goma, with 4 arriving at the INRB Goma and 3 others en route. 9 samples are still being held at the INRB Beni awaiting transport for shipment to the analysis laboratory.

RISK COMMUNICATION AND COMMUNITY ENGAGEMENT (RCCE)

- Dissemination of a message from the Military Governor of the Province regarding compliance with measures to combat the Plan communication plan currently being developed
 - Production of provisional communication timelines
 - Production of two official messages, one for preparedness and the other for response •
- Communication session for provincial government officials and staff during the parade morning;
- Briefing of Presicodesa on strengthening the SBC, CREC, reporting alerts and collecting community feedback;
 - Holding of the first meeting to analyze rumors and community feedback.
 - Plea to the Mayor of Bunia to prohibit the organization of wakes during this period of the Ebola response
 - Plea to the Sub-Proved EPST Bunia to relay the instruction from the PROVED prohibiting the expulsion of students for lack of wearing a mask.
 - Plea to the Commander of the Air Traffic Control Authority (RVA) of Bunia

INFECTION PREVENTION AND CONTROL

ZS Bunia

- 2 EDS completed and 1 in progress;
- Briefing of 5 emergency room care providers from HGR-Bunia on preventive measures;
- Provision of PPE and decontamination equipment for the practical session of the ZS Bunia teams;
- Decontamination of the isolation building at HGR-Bunia.
- Provision of some PCI inputs by the ALIMA partner

ZS Rwampara

- Briefing of the 4 staff members of the BALIDJA Morgue on the wearing of PPE before handling any body;
- Continued briefing of the 63 healthcare providers (Hygienists, Nurses, Doctors and Midwives) of CME Nyakunde) on the management of Biomedical Waste.
- 3 EDS completed and 1 in progress
- Decontamination of the isolation room at HGR/Rwampara;
- Briefing of 10 Surface Technicians and 15 PCI Supervisors on the port and removal of PPE.

ZS Mongbwalu

- Formative supervision in 4 health facilities including: Uponyaji Health Center, Rwampara Health Center, Amani Health Post and CS Tchunga (without doing the evaluation);

DPS North Kivu

- Decontamination of Matanda Hospital where the confirmed case from the Katwa health zone is staying;
- Matanda Hospital PCI score card evaluation: 70.6%;
- Creation of the case mapping;
- Status of the CTE for the release of PCI input needs (PPE, water supply, waste management, patient circuit).

LOGISTICS

- Development of the logistics needs report for ZS Goma, Katwa, Beni, Karisimbi, Mutwanga, Vuhovi, Kalunguta, Kamango, Mabalako, Oicha, Museniene, Kayina, Alimbongo, Butembo;
- Acquisition of Starling kits for internet connection from UNICEF;
- Receipt of 5 vehicles provided by the WHO;
- Receipt of 200 litres of diesel (Support from MEMISA-Belgium).

HOLISTIC CARE

- Briefing of service providers at HGR Bunia and Rwampara on the wearing of PPE;
- Clinical evaluation of patients;
- Delivery of 5 mattresses, 5 beds and other equipment to the CME
- Two isolation tents are being set up at the Bunia General Referral Hospital;
- Fitting out of a room at the Bunia General Referral Hospital is underway.
- Evaluation of the nutritional status of patients in isolation at the CME.
- 14 announcements of results;
- Awareness-raising of 97 family caregivers, 9 hygiene specialists, 11 workers, 92 teachers from CSJM and 25 taxi drivers;
- Family interviews with 4 families of deceased;

Table VI. Patient movement in healthcare facilities as of May 20

2026

Indicators		BUNIA						RWAMPARA					HGR	HGR	TOTAL
		HGR	SOFEPA	SOFEPA	RWAOLE	ERKYA	TOTAL	HGR	CME	SAHAMA	CS	TOTAL			
		6	4	0	12	6	28 beds	16,100	100,100		0	126 Beds	12	11	
Report		13	4	1	6	4	28	22	0	7	8	37	21	15,101	
News Admissions	PPL	0	0	0	0	0	0	0	2	3	0	5	0	1	6
	Others	6	0	1	3	2	12	2	6	4	0	12	9	0	33
	Total	6	0	1	3	2	12	2	8	7	0	17	9	1	39
Total Isolation		19	4	2	9	6	40			14	8	54	30	16	126
Outings	Healed	0	0	0	0	0	0	0	0	0	0	0	0	0	0
	Deceased	2	0	0	1	0	3	2	0	0	0	2	1	0	6
	No Case	0	0	0	0	0	0	0	5	8	0	13	0	0	13
	Escapees	1	0	0	0	0	1	0	1	0	0	1	2	0	4
	Transferred to HGR	0	0	0	0	0	0	0	0	0	1	1	0	0	1

SECURITY

- The security situation in the city of Bunia (Bunia and Rwampara health zones) is relatively calm
- Security briefing for deployed teams.

III. CHALLENGES

- Absence of standardized CT and
- CTE; Shortages of PCI equipment (PPE, DLM, chlorine) in North
- Kivu; Weak reporting of alerts by the zones;
- Insufficient financial resources to support all pillars of the response; Difficulty in transporting samples to laboratories in Goma and Kinshasa.
- Spread of rumors and proliferation of traditional treatment recipes.
- Lack of communication materials and tools.
- Lack of office supplies and equipment (computer tools); Insufficient vehicles for field teams to travel;

VI. Recommendation

- Make the CT and CTE facilities in Beni, Butembo and Goma operational (rehabilitation and circuit overhaul)
- Strengthen surveillance in order to raise alerts and search for contacts who have never been seen and have been lost to follow-up;
- Mobilize resources to support the different pillars of the response;
- Strengthen the analysis of rumors and community feedback and take action to address them
- Provide communication materials and tools

Provision of PCI Kits to the DPS Ituri



bed allocation to the ESS structure



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